

DEEP RESEARCH | 2026

Market Attractiveness & Competitive Landscape

Functional Uniform / Workwear / Medical Apparel B2B
Thailand & Southeast Asia

Research dossier for People's Garment Public Company Limited (PG)

Evidence cut-off: 2 September 2026

Evidence-led • Claim-level sources • No invented market figures

How to use this dossier

This pack is designed for selective extraction into the Megu report. Use the source codes [S01]–[S50] with the Source Register; retain the NPD and inference caveats when quoting conclusions.

1. Research boundary and evidence rules
2. Market size and growth: claimable vs non-claimable
3. Market-attractiveness assessment
4. Procurement behavior, contracts and reorders
5. Margin and value drivers
6. Competitive landscape
7. Barriers to entry
8. Certification and regulatory requirements
9. Organizational customer and buyer examples
10. Strategic implications and 90-day evidence plan
11. Appendices: NPD register and source register

Executive verdict — selectively attractive, not yet a proven large TAM. Public evidence supports a recurring, specification-led institutional niche across healthcare, manufacturing, hospitality and government. It does not support a current, definition-consistent market-size or CAGR claim for the combined segment in Thailand or SEA. PG's most defensible wedge is customized functional corporate/medical uniforms and selected reusable workwear, supported by fit-data, repeat-order service, branch-level fulfilment and product-performance proof. Commodity uniforms should be treated as price-led business; regulated surgical/isolation garments require a separate medical-device pathway.

1. Research boundary and evidence rules

1.1 Market definition used

This dossier separates four arenas because their economics and barriers are different:

12. **Basic corporate/service uniform:** polos, shirts, blouses, trousers, skirts and ordinary uniforms used for identity and appearance.
13. **Bespoke functional uniform:** corporate or medical apparel with comfort, stretch, moisture management, antibacterial, UV, visibility or similar performance claims, plus fitting and multi-location fulfilment.
14. **Industrial protective workwear:** garments selected against a real workplace hazard. ISO 13688 is a general protective-clothing base and must be paired with hazard-specific standards such as ISO 20471 for high visibility or ISO 11612 for heat/flame protection. [S29–S31]
15. **Medical apparel:** ordinary scrubs and nurse uniforms are not automatically medical devices. Surgical/isolation gowns or apparel marketed for protection against contamination may be medical devices, depending on intended use and national rules. [S26–S28, S32–S34]

Disposable nonwoven PPE, hospital linen and laundry services are discussed only where they affect competition or business-model economics; they are not treated as the same market as reusable uniforms.

1.2 Evidence hierarchy

Fact means government data, procurement documents, standards bodies or issuer filings. **Company disclosure** means a vendor's official website or annual report and is described as such. **Analyst inference** is an interpretation derived from cited facts and never presented as measured market data. **NPD** means not publicly disclosed or no credible definition-consistent public estimate was located.

Every numerical statement is tied to a source. Commercial market-report snippets with opaque methodology are excluded from the market-size conclusion.

2. Market size and growth: what can and cannot be claimed

2.1 Defensible conclusion

Thailand TAM/CAGR: NPD. SEA TAM/CAGR: NPD. No authoritative public series was located that consistently combines corporate uniforms, industrial workwear, ordinary medical scrubs and regulated medical garments. Government and ASEAN sources measure broader apparel, technical textiles,

healthcare, employment, tourism or project-level procurement. Those are useful demand proxies, but they cannot be added together or converted into a market value without assumptions that the sources do not disclose. [S01–S04, S17–S19]

Two figures that may appear attractive but should not be used as the target-market size are:

16. A Thailand Textile Institute article relays an Asia-Pacific medical-textiles outlook of USD 3 billion and 8.3% CAGR for 2020–2027. Its scope is broad medical textiles and the geography is Asia-Pacific, not Thailand B2B medical apparel. [S03]
17. Thailand’s 2018 medical-technical-textile exports were USD 59.86 million, up 7%, but the cited leading products were bandages/gauze, cellulose hygiene products and sutures—not uniforms or scrubs. [S04]

The correct report language is therefore: “No current public estimate with a matching definition was found; attractiveness is assessed from demand-base, procurement and competitor-economics evidence.”

2.2 Thailand demand-base proxies

Demand pool	Primary evidence	What it supports	What it does not support
Healthcare institutions	In 2024 Thailand had 1,404 inpatient hospitals/medical establishments—1,057 public and 347 private—and 174,336 beds. The system recorded 225.96 million outpatient visits and 11.47 million inpatients. [S05]	A large institutional base with repeated apparel use.	Market value, sets per worker, replacement rate or vendor share.
Private-hospital clinical workforce	Four reported full-time professions totalled 44,097 people: 9,314 doctors, 457 dentists, 3,131 pharmacists and 31,195 registered/technical nurses. [S05]	A directly relevant wearer pool for medical apparel.	Total staff, part-time staff or uniforms purchased per person.
National health workforce growth	Physicians rose from 36,472 in 2020 to 43,714 in 2024; registered nurses from 176,285 to 198,836. Derived CAGRs are about 4.6% and 3.1%, respectively. [S06]	Directional growth in wearer base.	Medical-apparel CAGR; the growth rates are analyst calculations, not source forecasts.
Manufacturing base	The 2021 manufacturing census recorded 438,259 establishments, 5.34 million persons engaged and 4.76 million employees. [S07]	Broad industrial workwear exposure.	Every worker needing functional or protective garments.
Current factory base	As of 2 February 2026, the Department of Industrial Works reported 73,984 factories and 4.07 million workers in its system. [S08]	A current administrative proxy for factory workwear demand.	National TAM or direct comparability with the census scope.
Ageing population	Thailand had 13.68 million people aged 60+ in 2024. [S09]	Structural direction for healthcare and care-work demand.	A direct apparel-spending forecast.

2.3 SEA demand-base proxies

Driver	Primary evidence	Market interpretation
Hospitality recovery	ASEAN visitor arrivals rose from 102.0 million in 2023 to 127.2 million in 2024, a derived increase of 24.7%, though still 11.5% below 2019's 143.7 million. [S17]	Supports renewed uniform demand in hotels, attractions, food service and transport. It is not a uniform-sales forecast.
Industrial investment	ASEAN FDI increased 8% to USD 226 billion in 2024; manufacturing represented 20% of FDI versus 9% in 2023. Manufacturing FDI rose in Cambodia, Malaysia, the Philippines and Thailand. [S18]	Supports industrial expansion and vendor opportunities. FDI is not proportional to headcount or garment spend.
Healthcare density	ASEANstats reports large differences in doctors and nursing/midwifery personnel per 10,000 people, ranging from high-density Singapore to lower-density Philippines and Indonesia. [S17]	Confirms healthcare apparel demand exists across the region but market maturity and account density differ by country. Reference years vary, so it is not a growth series.
Healthcare investment	Indonesia healthcare-sector FDI rose fivefold to USD 2.2 billion in 2024. [S19]	A directional signal for facilities and institutional procurement, not apparel revenue.

3. Market-attractiveness assessment

3.1 Segment attractiveness

Segment	Demand and rivalry	Differentiation / evidence	Attractiveness for PG
Basic corporate uniform	Recurring but exposed to budget delays, many private suppliers and price-led tenders. [S14, S38]	Low–medium barrier; design, fit and delivery help but sewing capacity is widely available. Margin: NPD.	Selective / defensive. Pursue only when volume, utilization or account access compensates for price pressure.
Bespoke functional corporate uniform	Recurring once patterns, sizes and branch routines are approved; rivalry includes Racé, TNLX and specialists. [S13–S15]	Medium–high barrier; wearer data, fitting, QC, fulfilment, reorder speed and performance proof create switching friction. Margin: NPD.	Most attractive wedge. Best match to PG's design-to-garment capability and current account base. [S12]
Ordinary scrubs / medical uniforms	Structural healthcare wearer base; competition from Racé, TNLX, PJ Garment and smaller suppliers. [S05–S06, S12]	Comfort, opacity, mobility, washing durability, fit and fulfilment matter. Ordinary scrubs are not automatically medical devices. Margin: NPD.	Attractive beachhead. Lower regulatory complexity than protective gowns and close to PG's 2025 initiative.
Industrial protective workwear	Demand is compulsory where hazards require protection; fewer credible providers when certification is specified. [S10]	High barrier by hazard; test reports, traceability and correct standards are essential. [S29–S31] Margin: NPD.	Selective option. Enter hazard-by-hazard, not through broad “functional” claims.

Segment	Demand and rivalry	Differentiation / evidence	Attractiveness for PG
Surgical/isolation gown and medical PPE	Institutional volumes can be large; specialist rivalry and import competition. [S15–S16]	High barrier; device classification, registration, QMS, barrier tests, labelling and potentially sterilization. [S26–S28, S32–S34] Economics differ from scrubs.	Option stage only. Separate business case and regulatory dossier required.

3.2 Geography prioritization — analyst inference

Thailand — first priority. PG already reports corporate/workwear clients in finance, government, state enterprises, private companies and education, and it expanded medical uniforms in 2025. Thailand also offers a visible hospital/factory demand base and procurement evidence. The constraint is strong domestic rivalry and lack of PG segment economics. [S05–S16, S35]

Singapore — selective, tender-led. GeBIZ provides transparent multi-year opportunities with firm and option periods, sample approval, embroidery and schedule-of-rates structures. Foreign suppliers can register, but individual opportunities may require Government Supplier Registration and financial standing. A local service partner is likely important for fitting and fulfilment. [S20–S23, S45]

Indonesia — partner-led scale option. Manufacturing and healthcare investment are attractive directional signals, but Trisula is an integrated incumbent and public e-procurement/local delivery requirements create access friction. [S18–S19, S39, S46]

Malaysia — opportunity with local-access constraints. A 2026 Hospital Kuala Lumpur tender illustrates that some opportunities can be restricted to Bumiputera firms registered with the Ministry of Finance under a specified field code. Medical PPE requirements are also explicit. Local partnering and SKU classification are prerequisites. [S24, S27, S33]

Vietnam and Philippines — selective account development. Both have national e-procurement and medical-device pathways. Public examples show short-cycle lump-sum orders as well as Class A gown registration, but no evidence supports an average contract size or cadence. [S25, S28, S46]

4. Procurement behavior, contracts and reorders

4.1 Observed buying patterns

Buying characteristic	Verified evidence	Commercial implication
Headcount and allocation per wearer	Bangkok's Ladprao district specified 476 workers × 2 sets = 952 sets, exact line items, onsite measurement and reference prices of THB 995–1,100 per set. [S13]	Order economics depend on wearer roster, size mix, fitting and changes during the contract—not only garment cost. Do not treat this case as a national price benchmark.
Detailed product specifications	A Thai public-hospital ICU tender specified fabric composition, thread count, at least nine stitches per inch, embroidery, exact sizes and retention of dimensions after the hospital wash program. [S15]	Wash durability, construction consistency and approved samples can be bid gates.

Buying characteristic	Verified evidence	Commercial implication
Recurring tender activity	Khon Kaen procurement pages list clothing-material tenders in 2022, 2023, 2024 and 2025. [S14]	Supports repeated buying events, but not proof of annual renewal with the same supplier.
Multi-year firm + option	Nanyang Polytechnic's 2026 scrub procurement uses a firm two-year period plus a two-year option; a culinary-uniform procurement also uses 2+2 years. [S20, S22]	Pricing, supply continuity and repeat production must hold across multiple years. Option periods are not guaranteed revenue.
Design + supply + extension option	Sentosa Development Corporation tendered design, supply and delivery for two years with a one-year extension option and a schedule of rates. [S21]	Design, annual replenishment pricing and service stability are part of the value proposition.
Sample qualification	A Singapore scrub quotation required physical samples within 10 working days after closing. [S20]	Sampling speed and controlled fabric/colour availability influence win probability and cost-to-bid.
Short lump-sum order	A Philippines provincial tender combined staff and patient garments in one goods lot with a 30-day delivery requirement. [S25]	The market also contains smaller, lumpy orders; a universal contract duration is not defensible.
Local registration restrictions	A Hospital Kuala Lumpur tender was limited to Bumiputera firms with specified Malaysia MOF registration; Singapore tenders may require GSR. [S24, S45]	Country-specific partner and vendor-registration strategy is a barrier to regional scale.

4.2 What is known about repeat orders

Racé publicly states that it retains each wearer's size/pattern for the life of a contract, offers onsite/mobile fitting, individual or branch-level sorting and delivery, and can reproduce garments immediately. These claims are not independent market averages, but they reveal the service standard a serious competitor markets to enterprise buyers. [S35–S36]

The strongest evidence-based interpretation is that switching friction is created by approved fabric and patterns, wearer-level size data, embroidery/brand files, quality acceptance and fulfilment routines. Replacement and new-employee orders can then be easier for the incumbent. However, **average reorder interval, renewal rate, churn, backlog and average contract value remain NPD** for PG and the Thai competitors reviewed.

4.3 Buyer power and cash conversion

Public tenders can use price criteria, require e-GP registration, financial capacity, performance security and defect correction. The BMA case required 5% contract security and remediation within seven days; applicable preferences for SMEs or Made in Thailand depend on current rules and tender conditions. [S13, S43–S44]

A transparent overseas corporate-uniform analogue reports 1–3 year price validity and typically 60-day customer credit, but it is a Hong Kong issuer—not proof of Thai or SEA averages. It should be used only as a working-capital risk analogue. [S50]

5. Margin and value drivers

5.1 What the public numbers support

There is **no reliable public industry gross-margin benchmark** for the exact Thailand/SEA segment. PG's 2025 company-wide gross margin was 21.40%, but the company does not disclose a uniform/medical segment margin. Thai Wacoal, TNLX, PJ Garment, FN and BTNC also do not publish comparable uniform-only margins. [S12, S35–S38]

The closest direct regional comparator is Trisula. Its 2024 uniform segment reported IDR 168.079 billion of sales, IDR 128.654 billion cost of sales and IDR 39.425 billion gross profit. The derived gross margin is 23.46%, versus 19.55% in 2023. Sales fell 2.99% while gross profit rose 16.38%. This demonstrates that mix, pricing, input cost and execution can matter more than volume, but 23.46% is **not a market average and not a PG forecast**. [S39]

Medtecs adds a different lesson: in 2024 its hospital-services gross profit rose after contract renewals lifted linen rates and consumption increased, while the company still recorded a large net loss partly associated with slow-moving PPE inventory provisions and expected credit loss. Service contracts can improve economics, but inventory and credit risk can overwhelm them. [S40]

5.2 Evidence-supported value drivers

18. **Performance proof:** hazard or use-case standards, wash durability, opacity, mobility, antibacterial or moisture functions must be demonstrable—not marketing adjectives. [S15, S29–S34]
19. **Fit and wearer data:** onsite measurement, mobile fitting, controlled size range, stored patterns and alteration/remake service reduce adoption friction. [S13, S35–S36]
20. **Repeat-order speed:** stable approved fabric, patterns, trims, embroidery files and size records shorten replenishment and new-hire fulfilment. [S35–S36]
21. **Multi-location fulfilment:** individual labelling, person/branch sorting and delivery solve procurement workload for buyers. [S36]
22. **Life-cycle cost:** industrial laundering performance, replacement rate and loss control can matter more than purchase price. ISO 15797 provides a test basis for industrial-laundering effects; service competitors also use rental/laundry/RFID models. [S29, S41]
23. **Traceability and sustainable procurement:** GRS chain of custody, Green Label, CoolMode, energy labels and Made in Thailand can differentiate where the TOR or buyer policy values them, but they are not universal legal requirements. [S11, S35, S43–S44]
24. **References and delivery reliability:** named institutional references, sample approval and multi-year option continuity create credibility barriers. [S20–S24, S35, S39]

5.3 Cost and margin drags

Custom SKUs, small and frequent lots, size complexity, samples, onsite fitting, alterations, individual packaging, branch delivery, warranty/remake, testing, tender preparation, performance security, long credit and safety stock all consume contribution margin and working capital. Large-volume orders can improve utilization, but PG itself says it collaborates on cost management for high-volume orders, indicating buyer power can offset scale benefit. [S12–S13, S35–S36, S50]

PG should calculate contract contribution margin rather than apply a market-average markup:

Contract contribution = invoiced revenue – fabric/trim – direct labour – subcontract testing/sterilization – sampling/fitting – alterations/remakes – individual/branch fulfilment – warranty/defect cost – tender cost – financing and inventory carrying cost.

No numeric premium should be assigned to antibacterial, moisture-management, flame-resistant, recycled or other features until quote-to-order and customer/SKU contribution data prove it.

6. Competitive landscape

6.1 Thailand — direct and conditional competitors

Company	Classification	Evidence-backed offer / position	Economics disclosure	Implication for PG
Racé / Thai Wacoal	Direct: corporate, hospital/nurse, industrial uniforms	Made-to-order/tender model; design and pattern teams; onsite/mobile fitting; 100% sewing QC; wearer/branch sorting; stored patterns; remake service; functional and sustainability labels. Publicly lists major hospitals, banks and corporates. [S35–S36]	Uniform-only revenue and margin NPD.	Closest disclosed service benchmark. PG must match account implementation, not just product design. Same Saha ecosystem creates both rivalry and partnership possibilities.
TNLX	Direct: corporate workwear/uniforms	Finance, petrochemical, education, automotive, healthcare/hospitals, hotels/spas and police uniforms. ICC acquired 70% in 2024. [S37]	Segment scale/margin NPD.	Strategic channel overlap: ICC represented 40.12% of PG sales but controls an overlapping uniform producer. This is an analyst inference of channel/buyer-power risk, not a disclosed conflict.
P.J. Garment	Direct: industrial, cleanroom/ESD, medical, hospitality and corporate	Claims two factories, over one million pieces annual capacity and MOQ 100 pieces/design, with separate sample/urgent and volume capability. [S38]	Private; revenue, utilization and margin NPD.	Flexibility and lower MOQ are a credible specialist benchmark; certificate scope must be verified.
BTNC / Uniform Specializer	Direct: corporate and professional/medical clothing	SET-listed issuer; FY2025 management said macro uncertainty delayed uniform purchase decisions and segment revenue declined significantly. [S14]	Segment amount and margin NPD.	Confirms budget cyclicality and rivalry within the Saha ecosystem.

Company	Classification	Evidence-backed offer / position	Economics disclosure	Implication for PG
FN Uniform	Direct but smaller/newer B2B channel	Corporate image design and budget-positioned uniforms; issuer reported a Q3/2025 milestone of THB 35 million in uniform sales. [S42]	Annual segment revenue and margin NPD.	Evidence of new listed-company participation; avoid overstating scale from one quarterly milestone.
Thai Hospital Products	Conditional direct: surgical/isolation gowns and disposable medical PPE	Manufacturer of medical disposables; publicly claims ISO 9001, ISO 13485, Thai GMP, cleanroom and AAMI/EN 13795 product capability. [S34]	Private; no audited segment economics.	Adjacent if PG stays in ordinary scrubs; direct specialist if PG enters protective medical garments.

6.2 SEA benchmarks and adjacent rivals

Company	Classification	Evidence	Implication
Trisula / Trisco / Mido	Direct regional benchmark	Integrated textile/garment/uniform group. Annual report names airline, airport, hospitality, food-service and healthcare clients and discloses uniform-segment economics. [S39]	Best transparent comparator for account mix and gross margin; not a forecast for Thailand.
Medtecs	Direct/adjacent medical apparel and hospital-textile services	OEM medical products, hospital linen/laundry/logistics; Philippines facility claims medical garments, workwear, uniforms, isolation gowns, ISO 9001/13485 and FDA registration. [S40]	Shows service-contract upside plus inventory/credit downside; specialist credentials raise the bar for protective medical apparel.
MBV / Oren	Adjacent corporate apparel	FY2024 consolidated revenue RM 200.929 million and gross margin 31.2% across imprintable apparel and gifts; Oren reports large ready stock and printing/embroidery. [S41]	Fast fulfilment and stock breadth compete with made-to-order suppliers; consolidated margin is not a uniform benchmark.
Hi-Tech Apparel	Adjacent scale/ODM benchmark	Claims 14,000+ staff, multi-country capacity, 40+ million garments/year, automation and 3D/digital product development. [S47]	PG should not compete primarily on general ODM scale; reported capacity figures are not fully consistent and are company claims.
Warrix	Adjacent brand/MTO benchmark	2025 made-to-order revenue THB 201.90 million, 11.87% of operating revenue, down a derived 36.2% from 2024. [S48]	Demonstrates MTO cyclicity but category is sports/team apparel, not medical/workwear.
Pan Brothers	Adjacent functional-apparel manufacturer	Produces technical, functional and activewear garments in Indonesia. [S49]	Manufacturing capability benchmark; no evidence supports classifying it as a direct institutional-uniform rival.

6.3 Competitive structure

Analyst inference: rivalry is bifurcated. Basic polos and office uniforms are fragmented, price-led and accessible to many suppliers. Enterprise bespoke and functional contracts have fewer credible contenders because the supplier must coordinate design approval, samples, wearer data, multi-size production, QC, branch fulfilment and repeat orders. Protective workwear and regulated medical apparel add product-testing, traceability, QMS and registration barriers. [S13–S16, S20–S39]

PG's disclosed strengths are integrated design/knitting/garment production, a 222-supplier pool, total capacity of 2.712 million pieces and 83.68% utilization in 2025, ISO 9001/14001 and GRS, and an existing corporate/workwear base. The capacity and margins are company-wide, not uniform-specific. [S12]

PG's evidence gaps relative to leading rivals are more important than its sewing capacity: no named reference clients; no uniform/medical revenue or margin; no bid-win rate, backlog, contract length or reorder rate; no published sample lead time, OTIF, alteration/return rate or employee-level ordering/size system; and no current product-level medical-PPE or workwear certificate scope. These should be treated as diligence items, not assumed weaknesses in actual operations.

7. Barriers to entry

7.1 Commercial and operational barriers

25. **Approved pattern, colour and fit system.** Samples, wearer measurements and retained patterns create implementation work that a new supplier must repeat. [S13, S20, S35–S36]
26. **Multi-size, small-lot and replenishment flexibility.** New hires and replacements create small, frequent orders even when the initial rollout is large. Flexible inventory and production scheduling are required; frequency is NPD. [S35–S36, S50]
27. **Service footprint.** Onsite fitting, alterations, individual labelling and multi-branch delivery require local operations or partners. [S35–S36]
28. **References and incumbent credibility.** Publicly listed accounts and prior tender performance reduce buyer risk; current contract status may not be public. [S35, S39]
29. **Tender administration and financial capacity.** Vendor registration, e-procurement, bid documents, performance security, financial standing and country-specific eligibility can exclude otherwise capable manufacturers. [S13, S24, S43–S46]
30. **Working capital.** Multi-year price commitments, safety stock, option periods and credit terms can create inventory and receivable risk. [S20–S22, S40, S50]
31. **Functional and regulatory proof.** Test reports must match the hazard and intended use; ISO 13688 alone is insufficient for a protective claim. Medical claims can trigger national device regulation. [S26–S34]

7.2 Barrier intensity by arena

Arena	Entry barrier	Main exclusion mechanism
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Arena	Entry barrier	Main exclusion mechanism
Basic corporate garments	Low–medium	Price, design acceptance, delivery and relationships.
Enterprise bespoke uniforms	Medium–high	References, wearer-data process, approved patterns, QC, fulfilment and continuity.
Industrial protective workwear	High for safety-critical use	Hazard-specific standard, test evidence, traceability and buyer audit.
Ordinary scrubs / nurse uniforms	Medium	Fabric opacity/comfort/wash durability, fit, hospital experience and fulfilment.
Surgical/isolation gowns	High	Medical-device classification/registration, QMS, barrier/sterility requirements and specialist manufacturing.

8. Certification and regulatory requirements

8.1 Decision path

Step 1 — Define intended use and claims. If the garment is only an ordinary corporate uniform or comfort scrub, do not market it as PPE or a medical device. If it claims protection against contamination, blood-borne pathogens, heat, flame, low visibility, chemicals or other hazards, the relevant regulatory and technical pathway changes. [S26–S34]

Step 2 — Identify the country and buyer TOR. The ASEAN Medical Device Directive provides a regional framework, but registration is national. Public buyers can add their own standards, labels, local-content and vendor-registration requirements. [S26, S43–S46]

Step 3 — Build a SKU-level evidence pack. Test reports, QMS certificates, material certificates, traceability, labelling and declarations must refer to the specific product and valid scope. A corporate-level ISO certificate does not prove a garment meets a hazard standard.

8.2 Requirement map

Use case	Relevant requirement / standard	Status and caution
General protective clothing	ISO 13688:2013	Covers ergonomics, innocuousness, sizing, ageing, compatibility, marking and information. Must be used with a specific protective standard. [S29]
High-visibility clothing	ISO 20471:2013	Covers colour, retroreflection, minimum areas and placement. Use when the hazard/TOR requires visibility. [S30]
Heat and flame	ISO 11612:2015	Performance requirements for clothing protecting against heat/flame. Select only for relevant hazards. [S31]

Use case	Relevant requirement / standard	Status and caution
Industrial-laundrying durability	ISO 15797:2017	Test basis for dimensional stability, colour, seam/crease appearance and related effects of industrial laundering. Actual laundry conditions should also be validated. [S29]
Thailand surgical/isolation gown	Thai FDA listing and 2020 specific standard notice	Thai FDA lists surgical/isolation gowns and coveralls as Class 1 listing devices; intended use matters. Lab testing may cover flammability, lint, liquid penetration, tensile and hydrostatic resistance. [S32]
Thailand device manufacturing QMS	Thai FDA GMP framework	From 3 July 2024, Class 1 manufacturers face Annex C minimum requirements; Classes 2–4 use GMP Annex A, ISO 13485:2016 or TIS 13485-2562, subject to transition provisions. Apply only after SKU classification. [S32]
Malaysia medical PPE	MDA/GD/0058	Non-surgical gowns: PB70 Level 1/2 or EN 13795; surgical/isolation gowns: PB70 Level 3 or recommended Level 4 or EN 13795; sterile products also EN 556 or equivalent, plus labelling. [S27]
Singapore medical gown	HSA intended-purpose and Class A pathway	HSA's Class A guide lists surgical gowns under Rule 4; dealer licensing applies even where Class A product registration is exempt. Confirm each product. [S28]
Philippines medical gown	FDA CMDN	Class A devices use Certificate of Medical Device Notification; FDA verification includes surgical gown examples. [S28]
Regional medical-device framework	ASEAN Medical Device Directive	Four risk classes, essential safety/performance, substantiated claims, labelling, traceability and national registration/authorized representative. [S26]
Recycled-content claim	Global Recycled Standard	Product/chain-of-custody and transaction-certificate requirements; GRS product claims require the applicable recycled-content threshold and other environmental/social/chemical controls. [S11]
Harmful-substance assurance	OEKO-TEX Standard 100	Independent testing/certification for harmful substances across textile and accessory stages. Verify certificate, class and scope. [S11]
Thailand green/local procurement	Green Label, CoolMode, Label No.5, Made in Thailand	Voluntary or tender/policy-specific differentiators, not blanket legal mandates. Check current certificate and TOR. [S35, S43–S44]

9. Examples of organizational customers and buyers

Named accounts below are examples of market participation, not proof of a current or exclusive supplier relationship unless the buyer's award says so.

Supplier / buyer source	Organizations or buyer examples	Evidence qualification
PG filing	Financial institutions, government agencies, state enterprises, private companies and educational institutions; no names disclosed. [S12]	High confidence in sector exposure; account names/values NPD.
Racé public portfolio	Phyathai–Paolo, Huachiew, Piyavate, BDMS, Bumrungrad, Vibhavadi, LH Bank, Federation of Thai Industries, Eastin Hotel, NC Property and DTAC. [S36]	Company-listed references; contract date/value/current status not disclosed.
Trisula annual report	McDonald’s Singapore, National Healthcare Group Singapore, American Airlines, Singapore Airlines, SilkAir, Marina Bay Sands, Changi Airport Group and Resorts World Sentosa. [S39]	Issuer-filed names; some are historical wins and current status may vary.
Thai public buyers	BMA Ladprao, Chum Phae Hospital, Khon Kaen provincial health procurement, 50th Anniversary Hospital and Chulalongkorn Hospital/Thai Red Cross. [S13–S16]	Buyer-issued procurement evidence; order scope and award details vary.
SEA public buyers	Nanyang Polytechnic, Sentosa Development Corporation, Hospital Kuala Lumpur and Province of Catanduanes. [S20–S25]	Direct tender evidence; not private-sector market averages.

10. Strategic implications for PG

10.1 Recommended market position

PG should compete as a **functional-uniform program operator**, not only a garment factory. The offer should combine approved fabric/function, design, individual size data, sampling, multi-location rollout, rapid replenishment, alteration/remake controls and auditable performance evidence.

The priority order supported by evidence is:

32. **Thailand bespoke corporate/workwear and ordinary medical scrubs** for accounts where PG already has channel access and can prove service-level execution.
33. **Selected industrial functional workwear** one hazard and one standard at a time, beginning with use cases where PG or its fabric partners already possess test capability.
34. **Singapore/SEA non-device uniforms via local partners and tenders**, using Thailand as the production base and local teams for fitting, registration and delivery.
35. **Protective medical apparel only as a separate option**, after product classification, QMS gap assessment, test plan, sterilization/supply-chain design and customer economics are complete.

10.2 Do not claim yet

36. “The Thailand/SEA market is worth X” or “growing Y%” for the combined segment.

37. “Functional uniforms have higher industry margins.”
38. “Contracts renew annually” or any universal reorder interval.
39. “All medical uniforms are medical devices” or “ISO 13688 certification alone proves protection.”
40. Current exclusive relationships with customer logos listed by competitors.
41. Uniform-segment economics derived from PG, Thai Wacoal, BTNC, FN or another company’s total margin.

10.3 90-day evidence plan before committing capital

Workstream	Minimum evidence	Decision test
Internal segment P&L	Revenue, gross profit and contribution by customer/SKU; fabric, labour, sample, alteration, branch-delivery, warranty and financing costs.	Identify contracts that create cash contribution after service cost, not merely factory utilization.
Commercial funnel	Named pipeline, tender count/value, bid/no-bid, win rate, reasons lost, sales cycle, option periods and expected call-off.	Demonstrate repeatable acquisition in at least two target verticals.
Reorder economics	Reorder share, months to first reorder, new-hire vs replacement mix, minimum economic batch, obsolete fabric/trim rate.	Prove incumbent advantage and working-capital burden.
Service operations	Sample lead time, first-pass approval, OTIF, measurement error, alteration/remake rate, branch-delivery accuracy and complaint closure.	Match or exceed Racé-style service benchmark.
Certification map	Intended use and claims by SKU; applicable law/TOR; current test reports and certificate scope; partner gaps.	No protective or medical claim without a complete evidence path.
SEA route to market	Country-specific partner, vendor registration, import/authorized-representative pathway, fitting/delivery model and tender shortlist.	Proceed country-by-country only where access cost and local service economics are viable.

11. Final market-attractiveness conclusion

The opportunity is credible because demand pools are institutional and recurring, buyers publish detailed specifications and multi-year options, and high-service incumbents invest in fit, data and fulfilment. It is not yet proven as a large or fast-growing market in monetary terms because the exact Thailand/SEA TAM, CAGR, market share and average contract economics are not publicly disclosed.

For PG, the strongest risk-adjusted thesis is not “medical apparel is a high-margin growth market.” It is: **PG may be able to convert existing apparel capabilities into sticky, repeatable institutional programs where product function, fit-data and fulfilment reduce the buyer’s total operational burden.** That

thesis becomes investable only after PG supplies customer/SKU contribution margins, pipeline conversion, reorder behavior, service KPIs and certification gaps.

Appendix A — NPD register

The following items were not publicly disclosed or could not be verified with a matching definition as of 2 September 2026:

42. Thailand and SEA market size/CAGR for the exact combined segment.
43. PG uniform/medical revenue, gross margin, contribution margin, client count/names, backlog, bid-win rate, contract length and reorder rate.
44. Average market contract value, replacement interval, annual sets per employee and payment terms.
45. Thai public/private procurement split, import penetration and competitor market shares.
46. Uniform-only revenue/margin for Racé/Thai Wacoal, TNLX, PJ Garment, BTNC and full-year FN.
47. Universal price premium or gross-margin uplift for antibacterial, cooling, recycled, high-visibility, flame-resistant or medical-protective features.
48. Current contract status for customer names shown in vendor portfolios.

Appendix B — Source register

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Research note: Sources were prioritized in the order government/regulator/standards body → public procurement document → issuer filing → official company page. Company portfolio and certification claims should be revalidated before an actual tender. No paywalled or opaque commercial market-size estimate was used as a market fact.